

HOSPITAL DISCHARGE SUMMARY

Patient: Sarah Mitchell

DOB: June 15, 1985 (Age: 40)

MRN: CTX-2024-001

Admission Date: February 18, 2026

Discharge Date: February 21, 2026

Length of Stay: 3 days

Attending Physician: Dr. Michael Thompson, MD

ADMISSION DIAGNOSIS:

Acute pneumonia, left lower lobe

DISCHARGE DIAGNOSIS:

Community-acquired pneumonia, resolving

HOSPITAL COURSE:

Patient was admitted through ED with fever, productive cough, and dyspnea. Chest X-ray confirmed left lower lobe infiltrate. Started on IV antibiotics (Ceftriaxone and Azithromycin). Oxygen supplementation initially required, weaned to room air by day 2. Repeat chest X-ray on day 3 showed improvement. Patient afebrile for 24 hours prior to discharge. Tolerating oral antibiotics well. Respiratory status stable.

PROCEDURES PERFORMED:

- IV catheter placement
- Oxygen therapy

DISCHARGE MEDICATIONS:

1. Amoxicillin-Clavulanate 875-125mg twice daily x 7 days
2. Azithromycin 250mg once daily x 4 days
3. Lisinopril 10mg once daily (home medication)
4. Ferrous Sulfate 325mg once daily (home medication)

DISCHARGE INSTRUCTIONS:

Activity:

- Rest for the next week
- No strenuous activity
- Return to work in 1 week if feeling well

Diet:

- Regular diet, increase fluid intake

Medications:

- Complete full course of antibiotics even if feeling better
- Resume all home medications

FOLLOW-UP APPOINTMENTS:

- Primary Care: Dr. Amanda Chen, MD in 1 week
- Repeat chest X-ray in 4-6 weeks to confirm resolution

RETURN TO ED IF:

- Fever >101 F, worsening shortness of breath, chest pain